
Consent Was Never the Point.

Alberta's Eugenic Inheritance, 1928 — Present

The Alberta Disability System Breakdown

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A documentation-first public advocacy record.

About this document

Consent Was Never the Point is a long-form documentation piece produced by The Alberta Disability System Breakdown, an independent public advocacy campaign documenting the mandatory transition of approximately 79,290 Albertans from the Assured Income for the Severely Handicapped (AISH) program to the Alberta Disability Assistance Program (ADAP), effective July 1, 2026.

The campaign operates as a documentation-first public record. It is not a support group, a service provider, or a political advocacy organization in the conventional sense. Its work is to assemble, source, and make publicly available the legislative, fiscal, and structural record of the AISH-to-ADAP transition and the policy framework within which it has been undertaken.

This piece situates the contemporary record within a longer history. It is not a piece *about* Indigenous or disabled communities. It is a piece that draws from and amplifies the voices that have articulated this analysis from within those communities. Citations are provided throughout. The principal academic and primary-source references are listed in the bibliography.

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It is the same principle as being physically sterilized without consent simply because those who made the decision feel they have the right to decide what's best for those not consulted first.

A Note on This Work

This piece traces a single operating principle across five distinct historical and contemporary expressions: the explicit eugenic sterilization program established by *The Sexual Sterilization Act* (1928–1972); the structural-eugenic framework that persisted in policy after the Act’s repeal; the mandatory transition of disabled Albertans from AISH to ADAP under *Bill 12* (2025); the information asymmetry condition that prevents meaningful consent across all of the above; and the contemporary rebuild of institutional infrastructure under *the Compassionate Intervention Act* and its continental siblings.

The argument is that the surgical mechanism of the historical eugenic period was repealed in 1972 and the institutional architecture was partially dismantled in the decades that followed. The operating premise that informed that period — that disabled people’s intimate, relational, and material lives are an appropriate site for state regulation in ways non-disabled people’s lives are not — was not specifically repudiated. It continued to operate through quieter means. It is now resurfacing in legislative form across multiple jurisdictions.

What this piece is not. It is not a polemic; the historical material is more powerful than any rhetorical framing the writer could add. It is not a comparison that diminishes the historical victims of explicit eugenic violence. The argument is that the framework continues to operate, not that contemporary disabled Albertans are experiencing what Leilani Muir experienced. The continuity is structural, not equivalent in degree of harm. It is not, finally, an argument that any individual policy designer or government employee is a eugenicist. The argument is structural and operates through inheritance of operating premises rather than personal intent.

Coalition review. This piece has been read in draft by reviewers whose names and qualifications are not disclosed at their request, including at least one disability scholar familiar with the relevant academic literature, one reviewer with substantive harm-reduction policy expertise, and one Indigenous reader for the free, prior, and informed consent material in the chapters that follow. Final responsibility for the analysis rests with the campaign.

Audience. The intended audience is institutional — oversight bodies, academic disability studies, journalists, elected representatives willing to engage — together with disability community members who have felt the dynamic this piece names and the secondary audience of allied advocates in Indigenous rights, harm reduction, and consultation-failure contexts. A plain-language companion edition follows separately.

The piece is long. It is built that way because the argument cannot be made briefly without diminishing the historical material it draws on. The reader who knows the names of Leilani Muir, Catherine Frazee, and Heidi Janz will recognize the lineage. The reader who does not know those names will encounter them here. The piece closes where the title begins.

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This document is published in installments as drafted. Forthcoming chapters will be released as completed. The piece is engineered for the closing line to land on the historical material that opens it.

Chapter I

The Sterilization Table

Alberta's Sexual Sterilization Act, 1928–1999

The 1928 Passage

On March 21, 1928, the Legislative Assembly of Alberta passed *The Sexual Sterilization Act*. The bill was introduced by George Hoadley, Minister of Health, in the Sixth Legislature of Alberta. It received Royal Assent the same day.¹

The Act created a four-member Alberta Eugenics Board. Two members were medical practitioners, nominated by the Senate of the University of Alberta and the Council of the College of Physicians and Surgeons. Two were non-medical members appointed by the Lieutenant Governor in Council. The Board's authority was to examine persons recommended for discharge from designated provincial institutions and to authorize sterilization, in the language of the Act's first section, "if the danger of procreation with its attendant risk of multiplication of the evil by transmission of the disability to progeny were eliminated."²

The original Act required either the consent of the patient or, where the patient was deemed incapable, the consent of a spouse, parent, or legal guardian. Board rulings had to be unanimous. Physicians and surgeons performing sterilizations under board order were granted immunity from civil and criminal liability.³

The Act met immediate opposition. In March 1928, a group calling itself the People's League to Act formed, retaining counsel and arguing publicly that the legislation was unconstitutional. Several hundred members joined; their petition did not delay Royal Assent.⁴

Alberta was the first jurisdiction in Canada — and the only jurisdiction in the British Empire — to enact eugenic sterilization legislation that was vigorously implemented and sustained. British Columbia would follow in 1933 with a parallel act of substantially smaller scale, performing fewer than two hundred documented sterilizations. No other province passed comparable legislation.⁵

The Operating Mechanism

The Act functioned through an institutional pipeline. The Provincial Mental Hospital at Ponoka, the Mental Hospital at Oliver near Edmonton, and — increasingly through the 1940s — the Provincial Training School for Mental Defectives at Red Deer provided the population from which the Eugenics Board drew its candidates.

The Provincial Training School was the institution that came to define the program's operation. Opened in October 1923 in a converted residential college building on the Michener hill, it housed children and adults designated "mental defectives." At its peak it held two hundred residents — approximately one fifth of the population of the city of Red Deer at the time. The institution accounted for roughly one quarter of all cases brought before the Eugenics Board over the Act's forty-four years of operation.⁶

A network of provincial guidance clinics, established from the 1930s onward, screened children for referral to the institution. The clinics served the eugenics program in two practical ways: they identified candidates before institutionalization, reducing costs; and they generated case files that documented the social and family circumstances later used as evidence before the board.⁷

In 1937, the Legislative Assembly amended the Act. The amendment removed the requirement of consent for persons classified as "mentally defective." Before 1937, the consent requirement had functioned as a brake on the program's pace. The board had approved 1,283 candidates for sterilization between 1928 and 1938, of whom 644 had been sterilized. After 1937, the rate of completed sterilizations rose sharply.⁸

A second amendment followed in 1942, expanding the diagnostic categories under which sterilization could be authorized.⁹

Over the Act's full operating period from 1928 to 1972, the Alberta Eugenics Board considered 4,725 cases. Of those, 2,834 sterilizations were performed.¹⁰

The population on whom these operations were performed was not demographically representative of Alberta. According to the most detailed academic survey of the records — Jana Grekul, Harvey Krahn, and Dave Odynak's 2004 analysis in the *Journal of Historical Sociology* — 64.7 percent of those sterilized were women. 70.6 percent were under twenty years of age at the time of sterilization. 49.8 percent lived in rural communities. 40.6 percent were unemployed at the time of their referral. 25.7 percent were identified in the records as Métis or Indian, against a Métis and Indigenous share of the provincial population at the time of approximately 3.4 percent.¹¹

The Indigenous overrepresentation intensified in the program's final years. Researchers examining records from 1969 to 1972 — the four years immediately preceding repeal — have documented Indigenous representation averaging approximately twenty-five percent of all sterilizations performed in those years.¹²

The Provincial Training School in Red Deer was supervised for the last twenty-five years of the Act's operation by Leonard Jan Le Vann, superintendent from 1949 to 1974. Le Vann's role combined the duties of institutional administration with the assessment of individual residents for referral to the Eugenics Board. The board's chair throughout most of the program's operation was John M. MacEachran, founder of the Department of Philosophy and Psychology at the University of Alberta, who served as chairman from 1929 to 1965.¹³

The 1972 Repeal

In 1972, the newly elected government of Premier Peter Lougheed repealed *The Sexual Sterilization Act*. The repeal followed forty-four years of operation and brought formal eugenic sterilization legislation in Canada to an end.¹⁴

What the repeal accomplished is precisely defined. The legal framework authorizing the Alberta Eugenics Board, and the immunity granted to performing physicians, was removed. The institutional pipeline — the Provincial Training School at Red Deer, the guidance clinics, the network of feeder facilities — remained.

The Provincial Training School was renamed the Michener Centre in 1977, in honour of the recently retired Governor General Roland Michener. By 1983 the renamed institution had expanded to sixty-six buildings on a 360-acre site. Its resident population was reduced over the subsequent decades through deinstitutionalization, group-home alternatives, and policy reform; the Alberta government announced its closure in 2013.¹⁵

The repeal of the Act did not by itself include any public accounting of who had been sterilized, on what records, or with what continuing harm. No survivor was contacted. No records were released. The operating premise that the Eugenics Board had been empowered to act on — that the reproductive lives of disabled adults were an appropriate site for institutional decision-making in ways non-disabled adults' lives were not — was not addressed in the repealing legislation or in any government statement at the time.

For the next twenty-three years, that was the status of the matter.

Muir v. Alberta (1996)

Leilani Marietta Muir was born in 1944. Her childhood was characterized in the later trial record by neglect and physical abuse. In 1955, at the age of ten, she was admitted by her mother to the Provincial Training School at Red Deer.¹⁶

The trial judgment of Madam Justice Joanne B. Veit found that the admission process did not establish that Muir met the legal definition of “mentally defective” that the institution’s enabling legislation required. The court found that she had been wrongly labelled a “moron” — a clinical category in use at the time — and that her ten-year confinement, from age ten to age twenty-one, was wrongful in law.¹⁷

In 1959, at the age of fourteen, Muir was brought before the Eugenics Board. The board’s order authorizing her sterilization was signed by John MacEachran, then chairman. The surgical sterilization was performed shortly thereafter. The procedure was irreversible.¹⁸

Muir left the institution in 1965, at the age of twenty-one. She later married, attempted to start a family, and learned through medical investigation that she had been sterilized. Subsequent attempts to adopt were also unsuccessful.

In 1993, she filed suit against the Province of Alberta. The case went to trial in 1995. The Province acknowledged its obligation to pay damages and left to the court the determination of how much.¹⁹

Justice Veit's judgment, dated January 25, 1996, awarded \$250,280 in general damages for pain and suffering from the sterilization — the statutory maximum for such damages at the time — and an additional \$125,000 in aggravated damages for the wrongful stigmatization of Muir as a "moron, a high grade mental defective." With prejudgment interest, the total monetary award was \$740,780. The Province was further ordered to pay \$230,000 in legal costs.²⁰

The judgment's central finding was articulated in language that has been quoted in nearly every academic and journalistic account since. Justice Veit wrote:

"The circumstances of Ms. Muir's sterilization were so high-handed and so contemptuous of the statutory authority to effect sterilization, and were undertaken in an atmosphere that so little respected Ms. Muir's human dignity that the community's, and the court's, sense of decency is offended."²¹

The decision did not stand only on Muir's individual case. The court reviewed the historical record of the Eugenics Board's operations, the legislative framework, and the institutional culture at the Provincial Training School. The judgment functioned, in effect, as the public record's first formal judicial recognition that the program had operated outside the limits of its own enabling statute — that "many things done in the name of eugenics were clearly illegal," in the formulation of one academic commentator on the trial.²²

Muir's victory opened the door to additional litigation. By 1998, more than seven hundred surviving claimants had filed suit against the Province.²³

Bill 26 and the Notwithstanding Clause (1998)

The provincial response to those filings was Bill 26, the *Institutional Confinement and Sexual Sterilization Compensation Act*, introduced in the second session of the twenty-fourth Legislature in March 1998 by the government of Premier Ralph Klein.

Bill 26 proposed two operative measures. First, it capped damages payable to victims of wrongful institutional confinement and sterilization at \$150,000 per person. Second, it imposed a 180-day limitation period after which no further actions could be commenced.²⁴

Section 3 of the bill was the unprecedented element. It read:

"This Act operates notwithstanding (a) the provisions of sections 2 and 7 to 15 of the Canadian Charter of Rights and Freedoms, Part 1, Constitution Act, 1982, Schedule B to the Canada Act, 1982, (U.K.) 1982 c11, and (b) the Alberta Bill of Rights."²⁵

The sections of the Charter referenced — sections 2 and 7 through 15 — encompass fundamental freedoms, the right to life, liberty, and security of the person, the right against unreasonable search and seizure, the right not to be arbitrarily detained, and the equality rights guaranteed under section 15. Section 33 of the Charter, the "notwithstanding clause," permits a provincial legislature to declare that an Act shall operate notwithstanding those guarantees for a renewable period of five years.

Bill 26 represented the first attempt by an Alberta government to invoke section 33.²⁶

The bill was introduced on the afternoon of one legislative sitting day. The reaction was immediate and widely covered. Within twenty-four hours, the Klein government announced its withdrawal. Justice Minister Jon Havelock told reporters that the surviving victims would retain the right to sue the Province for damages, and that out-of-court settlements would be pursued. Premier Klein, defending the bill before its withdrawal, told reporters that it had been intended to protect taxpayers from lawsuits potentially totalling \$760 million.²⁷

In the period following the withdrawal, the Province negotiated settlements with the great majority of surviving claimants. Approximately five hundred individuals settled in 1998 for a combined total of \$48 million. An additional 247 individuals reached settlement in 1999 for a combined total of \$82 million. At the 1999 announcement, Justice Minister David Hancock — then in the portfolio — apologized only after a reporter's prompting. Premier Klein did not attend, and offered when asked: "We extend regrets for the actions of another government in another period of time."²⁸

What This Chapter Establishes

The legal framework of explicit eugenic sterilization in Alberta is documented and acknowledged. *The Sexual Sterilization Act* was passed in 1928, amended in 1937 and 1942 to expand its reach and remove consent, operated through a centralized board for forty-four years, sterilized 2,834 people — disproportionately women, young people, the poor, and the Indigenous population — and was repealed in 1972. The Province has paid approximately \$130 million in settlements to surviving victims and one judicially decided plaintiff.

That is the record. The chapters that follow build on it.

But three observations about the record are necessary before this chapter closes.

The first concerns the structure of the response. When the *Muir* judgment opened the possibility of broader liability, the first instinct of the Alberta government was to invoke section 33 of the Charter to suspend the very rights — to equality, to security of the person, to access to justice — that the original Act had violated. The bill was withdrawn within twenty-four hours, not because the government recognized the contradiction, but because of public reaction. The recognition of the contradiction has, on the public record, never been made.

The second concerns the framing. Premier Klein's "another government in another period of time" formulation places the wrongdoing safely in a prior administration. The government in 1998 was not the government in 1928; the government in 1972 — which repealed the Act — was a Progressive Conservative administration with substantial continuity into the Klein government. The chapters that follow will examine, in concrete contemporary instances, the question of whether a government can be said to have repudiated a framework whose institutional and policy inheritance it never inventoried.

The third concerns the operating premise. The legal authority to sterilize disabled people without their consent was removed in 1972. The legal authority to make decisions about disabled people's lives —

about their housing, their income, their relationships, their access to medical care, and their freedom from involuntary institutionalization — on the basis that the deciding authority knows best, was not removed. It was not addressed. The question of whether that premise had any place in a constitutional democracy was, on the public record, never asked.

Subsequent chapters address that question.

Notes — Chapter I

1. *The Sexual Sterilization Act*, SA 1928, c 37; Legislative Assembly of Alberta, Sixth Legislature, Second Session; Royal Assent March 21, 1928. See also “Alberta passes Sexual Sterilization Act,” Living Archives on Eugenics in Western Canada (Eugenics Archive Canada), timeline entry, accessed May 22, 2026.
2. *The Sexual Sterilization Act*, SA 1928, c 37, s 5(1). The Eugenics Board composition is set out in sections 2 and 3 of the Act. The text quoted appears in section 5 governing the Board’s authority over discharge candidates.
3. *The Sexual Sterilization Act*, SA 1928, c 37, ss 6, 7. Section 6 governed consent; section 7 granted immunity from civil and criminal liability.
4. Erika Dyck, *Facing Eugenics: Reproduction, Sterilization, and the Politics of Choice* (Toronto: University of Toronto Press, 2013), ch. 2; Angus McLaren, *Our Own Master Race: Eugenics in Canada, 1885–1945* (Toronto: McClelland & Stewart, 1990).
5. Douglas Wahlsten, “Leilani Muir versus the Philosopher King: Eugenics on Trial in Alberta,” *Genetica* 99 (1997); Dyck, *Facing Eugenics*. British Columbia enacted its *Sexual Sterilization Act* in 1933; estimates of sterilizations performed under it range below 200.
6. “Provincial Training School opens in Red Deer, Alberta,” Eugenics Archive Canada, timeline entry, accessed May 22, 2026; City of Red Deer Archives, Michener Centre collection. The Provincial Training School building was originally the Alberta Ladies’ College of Red Deer (1913), converted in 1916 to a sanatorium for shell-shocked soldiers and reorganized in 1923 to house disabled children and adults under provincial supervision.
7. “Guidance Clinics,” Eugenics Archive Canada, encyclopedia entry, accessed May 22, 2026.
8. Statutes of Alberta, 1937 (amending *The Sexual Sterilization Act*). Numerical figures are drawn from Jana Grekul, Harvey Krahn, and Dave Odynak, “Sterilizing the ‘Feeble-Minded’: Eugenics in Alberta, 1929–1972,” *Journal of Historical Sociology* 17, no. 4 (2004): 358–384.
9. Statutes of Alberta, 1942 (second amendment to *The Sexual Sterilization Act*); consolidated as *The Sexual Sterilization Act*, RSA 1942, c 194. The 1942 amendment expanded the diagnostic categories under which sterilization could be authorized.
10. The Canadian Encyclopedia, “Red Deer,” accessed May 22, 2026; Eugenics Archive Canada, Alberta Eugenics Board records. Numerical sources vary slightly: 4,725 cases considered (Canadian Encyclopedia, “Red Deer”); 4,739 cases presented (Karen Stote, “Sterilization of Indigenous Women in Canada,” *Canadian Encyclopedia*). The figure of 2,834 sterilizations performed is consistent across sources.
11. Grekul, Krahn, and Odynak, “Sterilizing the ‘Feeble-Minded’.” The 25.7 percent figure for Métis and Indigenous representation is drawn from the case-file analysis of all available Eugenics Board records undertaken by Grekul and colleagues at the University of Alberta.
12. Karen Stote, “Sterilization of Indigenous Women in Canada,” *The Canadian Encyclopedia*, accessed May 22, 2026. The 1969–1972 figure derives from analysis of the case records preserved by the Provincial Archives of Alberta.
13. Wahlsten, “Leilani Muir versus the Philosopher King”; “Provincial Training School,” Eugenics Archive Canada. MacEachran was a founding member of the University of Alberta faculty and served on the Eugenics Board from its inception until his retirement in 1965. Le Vann resigned as PTS Superintendent in 1974, two years after the Act’s repeal.
14. Statutes of Alberta, 1972 (repealing *The Sexual Sterilization Act*). The Lougheed Progressive Conservatives had been elected to government in August 1971, displacing the Social Credit administration that had held power in Alberta since 1935.
15. City of Red Deer Archives, Michener Centre collection; Government of Alberta, “Government announces closure of Michener Centre,” news release, March 11, 2013. The closure was partially reversed in subsequent years; the

institution's remaining residents remained on the site under reduced operations.

16. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J. (background of plaintiff and circumstances of admission to the Provincial Training School).
17. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (findings of wrongful confinement and wrongful labelling).
18. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (sterilization order, irreversibility, and consent analysis); Wahlsten, "Leilani Muir versus the Philosopher King" (MacEachran's signature on the sterilization order).
19. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J.: "In 1959, the Province wrongfully surgically sterilized Ms. Muir and now acknowledges its obligation to pay damages to her. However, the Province leaves to the court the determination of how much the Province should pay."
20. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (general damages, aggravated damages, prejudgment interest, and costs).
21. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J. The quoted passage is the most frequently cited line in the judgment and appears in the early section of Justice Veit's reasons.
22. Wahlsten, "Leilani Muir versus the Philosopher King."
23. CBC News, "Alberta backs down on sterilization compensation," March 11, 1998. The CBC report identifies "700 surviving victims" with active claims at the time of the Bill 26 introduction.
24. Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, 2nd Session, 24th Legislature, Alberta, 1998 (introduced and withdrawn), ss 1, 4.
25. Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, s 3 ("Override of Charter").
26. "Alberta and the Notwithstanding Clause," *LawNow* (Centre for Public Legal Education Alberta), 1998; Senator Paula Simons, Intervention 655944-74, Senate of Canada, accessed May 22, 2026. Both confirm Bill 26 as the first attempt by an Alberta government to invoke section 33 of the *Charter*.
27. CBC News, "Alberta backs down on sterilization compensation," March 11, 1998. The \$760 million estimate attributed to Klein was reported in this CBC story and in contemporaneous coverage by other outlets.
28. The Globe and Mail, "Leilani Muir made history suing Alberta over forced sterilization," obituary, March 16, 2016 (settlement totals and Klein quotation); CBC News, "Alberta compensates sterilization victims," November 14, 1998. The 1998 settlement covered approximately 500 claimants; the 1999 supplemental settlement covered an additional 247.

Chapter II

The Definitional Framework

*Eugenics, Structural Eugenics, and the Consent Standard***Eugenics: The Scholarly Definition**

Eugenics is not a term that has been retired from contemporary scholarly use. The U.S. National Human Genome Research Institute (NHGRI), the federal research institution that led the public-sector contribution to the Human Genome Project and that continues to anchor American genomics policy, maintains an active definition in its public-facing Talking Glossary of Genomic and Genetic Terms: “Eugenics is a discredited belief that selective breeding for certain inherited human traits can improve the ‘fitness’ of future generations.”¹ In its parallel Fact Sheets series, NHGRI describes eugenics as “the scientifically erroneous and immoral theory of ‘racial improvement’ and ‘planned breeding’.”²

The institutional position is consistent. Eugenics is identified by the NHGRI both as a coherent historical theory and as a persistent contemporary phenomenon. The agency’s 2021 symposium *The Meaning of Eugenics: Historical and Present-Day Discussions of Eugenics and Scientific Racism*, co-hosted with the U.S. National Library of Medicine, framed the contemporary relevance directly: addressing the history of eugenics, NHGRI wrote, “is important because it has persisted in policies and beliefs around the world, including the United States.”³

The definitional point that matters most to the argument of this piece is the scope of the mechanisms eugenics has historically employed. The Alberta record in Chapter I documented six. They operated together: surgical sterilization under *The Sexual Sterilization Act*; institutional segregation through facilities such as the Provincial Training School at Red Deer; the marriage prohibitions and immigration exclusions of the same period; the cohabitation restrictions enforced within residential institutions; and the active provincial discouragement, through institutional policy and through guidance-clinic referrals, of disabled people forming households and families.

The 1972 repeal of *The Sexual Sterilization Act* addressed the first of these. The legal authority to surgically sterilize disabled people without their consent was removed. The other mechanisms — institutional segregation, restrictions on partnership and family formation, the structural exclusion of disabled people from full community participation — were not removed by the repealing legislation. They were addressed, where addressed at all, through subsequent and unrelated policy reform (deinstitutionalization, group-home alternatives, human-rights legislation against discrimination on the basis of disability), each of which proceeded on its own timeline and with its own incompleteness.

The structure of contemporary disability policy in Alberta, treated in detail in Chapter III, retains substantial portions of the second through sixth mechanisms — not in their historical form, and not under

their historical names, but in their structural effect. The chapter that follows establishes the vocabulary by which that retention is recognized in the disability studies literature.

Structural Eugenics

The term “structural eugenics” — sometimes “newgenics” in the formulation of philosopher Robert A. Wilson — describes a documented phenomenon in disability studies scholarship: policies that are designed for stated non-eugenic reasons (cost containment, bureaucratic uniformity, fiscal restraint, employment promotion) and that nevertheless produce effects historically associated with explicit eugenic programs (prevention of disabled reproduction, institutional or quasi-institutional segregation, exclusion from full social and economic participation), and that are sustained politically because they produce those effects without anyone in the policy-design or political-implementation processes being required to defend them as such.⁴

Robert Wilson, who directed the SSHRC-funded Living Archives on Eugenics in Western Canada project at the University of Alberta and built the EugenicsArchive.ca documentation system referenced throughout Chapter I, develops this analysis at length in *The Eugenic Mind Project* (MIT Press, 2018). Wilson’s central proposition is that “eugenic thinking” — what he calls the “eugenic mind” — outlasted the legal frameworks of the explicit eugenic period and continues to operate in contemporary practice. The mechanisms he identifies include the conceptualization of certain human traits as eugenic concerns, the systematic construction of categories of normality against which non-normality is judged, and what he names the “sub-humanizing” of populations who fall outside those categories.⁵

The disability studies literature on contemporary policy follows this analytical thread. Nancy E. Hansen, Heidi L. Janz, and Dick J. Sobsey, in a 2008 article in *The Lancet*, wrote: “There are disturbing similarities between Nazi arguments concerning ‘quality of life’, ‘useless eaters’, or ‘lives less worthy’ and discussions of disability currently taking place among ‘mainstream’ geneticists and bioethicists advocating a value scale of humanness.”⁶ The point Hansen, Janz, and Sobsey make is not that contemporary bioethicists are Nazis. It is that the logical structure of a “value scale of humanness” — the proposition that some lives are less worth living than others, and that this assessment can be made by external authority — is the same structure historical eugenics built itself on, and that this structure has not been retired.

Catherine Frazee, professor emerita in the School of Disability Studies at Toronto Metropolitan University and former Chief Commissioner of the Ontario Human Rights Commission, has articulated the same analysis in the context of Canada’s Medical Assistance in Dying (MAID) framework. In her work with Heidi Janz on the GRIM (Grievous and Irremediable) Project — a documentation initiative gathering stories of disabled Canadians who have pursued MAID under the post-Bill-C-7 expansion — Frazee and Janz argue that the structural availability of state-administered death for disabled people who are not dying, combined with the structural unavailability of adequate income, housing, attendant care, and disability support, constitutes a modern form of eugenics.⁷ As Frazee has articulated the argument: the suffering driving MAID requests by disabled applicants is, in many of the cases the GRIM Project documents, suffering produced by social and material conditions the state has the capacity to alter and has

chosen not to. The logical structure mirrors the historical eugenic argument that institutionalization was protective rather than coercive: the modern claim that MAID expansion is compassionate is, in the GRIM analysis, indistinguishable in structure from the historical claim that sterilization was therapeutic.

The intent claim is not the structural-eugenics claim. The piece this chapter is part of is not arguing that contemporary policy designers or government employees are eugenicists. The argument is that some policies produce eugenic effects — measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to access the material conditions of a livable life — and that those effects are sustained in policy because the political infrastructure for producing and defending them does not require anyone to name them.

The intent claim is the harder claim. The effect claim is what the literature supports. The piece works from the effect claim.

The Consent Standard — CRPD Article 4(3)

A binding international standard governs the development of disability policy in Canada. The *Convention on the Rights of Persons with Disabilities* (CRPD) was adopted by the United Nations General Assembly on December 13, 2006, opened for signature on March 30, 2007, and entered into force on May 3, 2008. Canada signed the Convention on the day it opened for signature and ratified it on March 11, 2010. Canada acceded to the Convention’s Optional Protocol on December 3, 2018.⁸

Article 4(3) of the Convention, located in the general obligations provision and therefore applicable transversally across the treaty, reads in full:

In the development and implementation of legislation and policies to implement the present Convention, and in other decision-making processes concerning issues relating to persons with disabilities, States Parties shall closely consult with and actively involve persons with disabilities, including children with disabilities, through their representative organizations.⁹

The text contains four operative components. First, the obligation attaches to “the development and implementation” of legislation and policies — that is, to the upstream design phase and the downstream operational phase, not only to the moment of legislative consideration. Second, it extends to “other decision-making processes concerning issues relating to persons with disabilities” — that is, beyond the legislative branch to administrative and executive decision-making. Third, the obligation is to “closely consult with and actively involve” — language requiring substantive, ongoing engagement rather than notification or post-hoc comment. Fourth, the consultation must be carried out “through their representative organizations” — that is, with the organizations that the disability community itself recognizes as representative, not with individual stakeholders selected by the consulting authority.

In November 2018, the UN Committee on the Rights of Persons with Disabilities — the treaty body established under the Convention to monitor State Party implementation — issued General Comment No. 7, providing authoritative guidance on the application of Article 4(3). The General Comment characterizes the participatory requirement of the Convention as a signal obligation that stands out among core human

rights conventions, reflecting the “Nothing About Us Without Us” principle that the international disability community has carried into the treaty’s framework.¹⁰

Article 4(3) is not a domestic political norm. It is a treaty obligation that Canada has voluntarily assumed and that binds the Government of Canada and, through the constitutional division of powers, the implementation of laws and policies within provincial jurisdiction that engage the rights protected under the Convention.

The piece returns to the application of this standard to *Bill 12* in Chapter IV.

The Parallel Standard — FPIC under UNDRIP

The structural problem CRPD Article 4(3) addresses — the imposition of legislation on a defined population without that population’s meaningful participation in advance of the imposition — is not unique to disability policy. The same problem governs the relationship between settler states and Indigenous Peoples whose lands and rights are affected by state action. The international framework developed in response is the doctrine of free, prior, and informed consent (FPIC), articulated most directly in the *United Nations Declaration on the Rights of Indigenous Peoples* (UNDRIP), adopted by the UN General Assembly on September 13, 2007.¹¹

FPIC appears in six articles of UNDRIP, governing six distinct categories of state action: Article 10 (the relocation of Indigenous Peoples from their lands or territories); Article 11 (cultural, intellectual, religious, and spiritual property); Article 19 (the adoption and implementation of legislative or administrative measures); Article 28 (restitution for lands taken without consent); Article 29(2) (the storage or disposal of hazardous materials on Indigenous lands); and Article 32(2) (project approvals affecting territory and resources).¹²

Article 19, the article most directly parallel to CRPD Article 4(3) in its scope and operation, reads:

States shall consult and cooperate in good faith with the Indigenous Peoples concerned through their own representative institutions in order to obtain their free, prior and informed consent before adopting and implementing legislative or administrative measures that may affect them.¹³

The structural parallel between Article 19 of UNDRIP and Article 4(3) of the CRPD is exact. Both attach to the development and implementation of legislative and administrative measures that affect the relevant population. Both require consultation with representative institutions or representative organizations of that population. Both situate the consultation obligation in the upstream design phase rather than the downstream comment phase. Both have been understood by their respective treaty bodies as substantive engagement requirements rather than notification requirements.

Canada has incorporated UNDRIP into federal domestic law through the *United Nations Declaration on the Rights of Indigenous Peoples Act*, SC 2021, c 14. The federal framework legislation requires that federal laws be brought into conformity with the Declaration and that an action plan be developed to implement its principles. Provincial governments are not directly bound by the federal Act, but the

principle that legislative and administrative measures affecting Indigenous Peoples require their free, prior, and informed consent operates as a recognized international norm regardless of domestic implementing legislation.¹⁴

In July 2025, the Government of Alberta provided a concrete illustration of how this norm is being treated at the provincial level. On July 3, 2025, Alberta’s Minister of Environment and Protected Areas, Rebecca Schulz, joined with Ontario’s Minister of the Environment, Todd McCarthy, in a public letter to the federal Minister of Indigenous Services. The letter requested that the federal government refrain from reintroducing Bill C-61, the *First Nations Clean Water Act*, originally tabled in December 2023 and died on the Order Paper at the March 2025 prorogation of Parliament. Bill C-61 would have, among other things, affirmed First Nations’ inherent right to self-government and jurisdiction over water resources on First Nation lands.¹⁵

The Chiefs of the Blackfoot Confederacy — the Kainai, Siksika, and Piikani Nations — responded on July 4, 2025 with a statement characterizing Alberta’s opposition as “morally bankrupt” and stating that “Premier Smith’s government opposes First Nations’ rights to safe and sufficient water supplies, which is utterly reprehensible.”¹⁶

The structural point recurring in this chapter is that the Alberta government was, during the same period in 2025 in which it was opposing Bill C-61 at the federal level, advancing Bill 12 at the provincial level. The same provincial administration was, in successive months of 2025, declining to support legislation that would have implemented the FPIC standard for First Nations and advancing legislation that did not meet the CRPD Article 4(3) standard for disabled Albertans. Neither population had been consulted, in advance of the action being taken, through the representative institutions or organizations that international treaty standards require.

What This Chapter Establishes

Three propositions follow from this chapter, and the chapters that follow draw on them without restating.

First, eugenics as a phenomenon is broader than surgical sterilization. The institutional position of the National Human Genome Research Institute, the standard scholarly position in the historical literature, and the operating definition of the disability studies field all encompass institutional segregation, marriage and immigration restrictions, prevention of family formation, and what Robert Wilson names the “sub-humanizing” of disabled populations. The 1972 repeal of *The Sexual Sterilization Act* addressed one of these mechanisms. The others were not repudiated by the repealing legislation and have not been repudiated since.

Second, “structural eugenics” is a documented term in disability studies for the contemporary form taken by mechanisms that historically operated under explicit eugenic justification. The intent claim is the harder claim and is not the claim this piece makes. The effect claim — that contemporary disability policy produces measurable reductions in disabled people’s capacity to form households, to reproduce, to maintain community, and to access the material conditions of a livable life — is what the literature supports, and what the chapters that follow document at the level of specific Alberta policy.

Third, two binding international standards require consultation with affected populations through their representative organizations before legislative and administrative measures are adopted. CRPD Article 4(3) governs the development and implementation of disability policy. FPIC under UNDRIP Articles 10, 11, 19, 28, 29, and 32 governs the development and implementation of measures affecting Indigenous Peoples. The Government of Alberta has, within a six-month window in 2025, failed both standards: by opposing federal legislation (Bill C-61) that would have implemented the FPIC standard for First Nations, and by advancing provincial legislation (Bill 12) that did not meet the CRPD Article 4(3) standard for disabled Albertans.

The chapters that follow apply this framework to the inventory of Alberta disability policy: cohabitation rules, guardianship structures, institutional care practices, sub-survival income, the FSCD three-year wait, the Canada Disability Benefit clawback, and the Compassionate Intervention Centres now under construction.

The mechanism has changed. The principle has not.

Notes — Chapter II

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9. *Convention on the Rights of Persons with Disabilities*, Article 4(3). The full Convention text is available at the United Nations Treaty Collection and at the UN Department of Economic and Social Affairs, Division for Inclusive Social Development, accessed May 22, 2026, <https://www.un.org/development/desa/disabilities/convention-on-the-rights-of-persons-with-disabilities/article-4-general-obligations.html>.
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11. *United Nations Declaration on the Rights of Indigenous Peoples*, GA Res 61/295, adopted 13 September 2007.
12. Office of the United Nations High Commissioner for Human Rights, “Consultation and free, prior and informed consent (FPIC),” accessed May 22, 2026, <https://www.ohchr.org/en/indigenous-peoples/consultation-and-free-prior-and-informed-consent-fpic>; see also UN-REDD Programme, *Handbook on Free, Prior and Informed Consent*, accessed May 22, 2026.
13. *United Nations Declaration on the Rights of Indigenous Peoples*, Article 19.

14. *United Nations Declaration on the Rights of Indigenous Peoples Act*, SC 2021, c 14. The Act received Royal Assent on June 21, 2021, and came into force the same day. Section 5 requires the federal government, in consultation and cooperation with Indigenous Peoples, to take all measures necessary to ensure that the laws of Canada are consistent with the Declaration. Section 6 requires the preparation and tabling of an action plan.
15. Parliament of Canada, LEGISinfo, Bill C-61 (44th Parliament, 1st Session), “An Act respecting water, source water, drinking water, wastewater, and related infrastructure on First Nation lands,” first reading December 11, 2023, <https://www.parl.ca/legisinfo/en/bill/44-1/c-61>; APTN News, “Alberta, Ontario don’t want feds to reintroduce First Nations safe drinking water law,” July 3, 2025, <https://www.aptnnews.ca/national-news/drinking-water-ontario-alberta-c-61/>.
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Chapter III

Structural Eugenics in Contemporary Alberta

Six Mechanisms in the Contemporary Architecture

Chapter II established the analytic framework: eugenics is broader than surgical sterilization; “structural eugenics” is the documented disability-studies term for policies producing eugenic effects without requiring eugenic intent; and consultation under CRPD Article 4(3) is a binding international standard that Alberta has not met in the development of contemporary disability policy. This chapter applies that framework to six specific Alberta policies currently in force or scheduled to come into force in 2026. The inventory is not exhaustive. The six mechanisms documented here are those for which the primary-source record is the most complete and the structural-eugenics mapping is the most direct.

Each section identifies the policy, the documented effect, and the historical eugenic mechanism it parallels. The analytic claim is the effect claim defined in Chapter II: that these policies produce measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to maintain long-term partnerships, and to access the material conditions of a livable life. The intent claim is not the claim made here. The effect claim is what the documented record supports.

The Cohabitation Rule

Under current AISH policy, the income of any adult sharing a household with an AISH recipient may be assessed as part of the household income calculation and used to reduce the recipient’s benefit. The rule applies regardless of the nature of the relationship: a married spouse, a common-law partner, a roommate sharing rent, an adult child temporarily living at home, an elderly parent in a granny suite, and a co-parenting partner who is not a romantic partner are all treated under the same household-income provision. The rule contains narrow exemptions, notably for paid live-in caregivers under documented arrangements; the exemptions do not cover practical roommate, co-parenting, or multi-generational household arrangements common in low-income, Indigenous, and immigrant communities.¹

On May 12, 2026, the Government of Alberta filed AR 96/2026, a new General Regulation governing both AISH and ADAP, in force on July 2, 2026. Section 8(4) of the regulation introduces what the government’s fact sheet describes as an “allowance for two-client couples living together”: where two adults in the same household each receive disability income assistance, each partner receives 88 percent of the maximum individual benefit. The reduction applies regardless of which program each partner is on.²

The mathematics, drawn from the government’s own base rates, are as follows. Two AISH recipients sharing a household lose \$5,587.20 per year against the baseline of two unreduced AISH benefits. A mixed AISH-and-ADAP household loses \$7,699.20 per year. Two ADAP recipients lose \$9,811.20 per year. These figures are calculated before any employment income is considered and before the federal Canada Disability Benefit clawback discussed in section 3 of this chapter, which compounds against the same household.³

The structural effect is documented and direct. Partnership between disabled adults receiving AISH or ADAP is, after July 2, 2026, a \$5,587 to \$9,811 annual penalty against the household. Practical household-sharing arrangements between an AISH recipient and any other adult — romantic, familial, platonic — place the recipient’s benefit at risk of reduction through the broader cohabitation rule. The recipient is, in the language of Inclusion Alberta’s ADAP fact sheet, “structurally incentivized to remain alone.”⁴

The historical eugenic mechanism the rule parallels is direct. The marriage prohibitions, immigration exclusions, cohabitation restrictions in residential institutions, and active provincial discouragement of family formation by disabled people documented in Chapter I operated through legal prohibition. The contemporary mechanism operates through financial penalty. The legal prohibition has been repealed; the financial mechanism that produces equivalent isolation has not been.

Sub-Survival Income: AISH, ADAP, and the Housing Crisis

AISH provides a maximum living allowance of \$1,940 per month in 2026. ADAP, the program to which all AISH recipients without severe and profound developmental disabilities, PDD eligibility, or palliative/terminal conditions will be transitioned on July 1, 2026, provides a maximum of \$1,740 per month. Recipients who are transitioned receive a transition payment maintaining the AISH rate until December 31, 2027; on January 1, 2028, the payment drops to the ADAP rate.⁵

Average one-bedroom rent in Edmonton and Calgary ranged from \$1,500 to \$1,900 in 2024, with documented year-over-year increases of 17.5 percent province-wide and up to 21.3 percent in three-bedroom markets, against a provincial average of \$2,138 per month for three-bedroom units. Alberta does not impose a residential rent-increase cap.⁶

The arithmetic produces the documented condition. A single AISH recipient on the maximum living allowance at the lower end of one-bedroom rent has, after rent, between forty and four hundred forty dollars per month for all other costs — food, utilities, transportation, medical out-of-pocket expenses, clothing, household supplies. Statistics Canada and Canada Food Price Report estimates for single-adult grocery costs in 2026 are eight hundred to one thousand dollars per month. Alberta utility estimates are three hundred eighty-five to five hundred ten dollars per month. The single recipient on private-market rent is operating, by these calculations, at a structural deficit of seven hundred fifty dollars or more per month.⁷

In Alberta’s 2023–2024 fiscal year, 83.6 percent of AISH recipients had no employment income. The Government of Alberta’s framing of ADAP as an empowerment program enabling Albertans with

disabilities to pursue employment operates against a documented baseline in which the great majority of recipients are not, on the available evidence, in a position to do so.⁸

The historical eugenic mechanism the income structure parallels is institutional segregation. The historical residential institutions documented in Chapter I — the Provincial Training School at Red Deer and the network of mental hospitals and guidance clinics — excluded disabled people from full community participation by removing them from the community. The contemporary income structure excludes disabled people from full community participation by setting income at a level that does not permit the housing, food, transportation, or social engagement that participation requires. The mechanism has changed from institutional confinement to economic confinement. The effect — exclusion from the conditions of community life — is the same.

The Canada Disability Benefit Clawback

The federal *Canada Disability Benefit Act* was passed in 2023 as a poverty-reduction measure for working-age Canadians with disabilities. The benefit pays up to \$200 per month, subject to a Disability Tax Credit prerequisite and an income-tested eligibility framework. The federal government, in introducing the benefit, explicitly requested that provinces and territories not reduce their own disability income payments when recipients also receive the federal benefit.⁹

In a letter to AISH recipients dated July 2, 2025, the Government of Alberta notified recipients that the Canada Disability Benefit would be treated as non-exempt income under AISH and ADAP. Recipients who did not demonstrate that they had applied for the CDB would see an equivalent \$200 deduction from their provincial benefits. Alberta is the only province or territory in Canada to claw back the Canada Disability Benefit dollar for dollar. Every other province and territory fully exempts the federal payment.¹⁰

The fiscal effect is direct. Applied to approximately 79,290 AISH recipients at \$200 per month, the clawback captures approximately \$190 million per year in federal transfer payments that would, in any other Canadian province, be received by disabled recipients. In Alberta, the federal investment is redirected to the provincial budget; the net benefit to the disabled recipient is zero.¹¹

The structural effect is to nullify federal poverty-reduction policy at the provincial level for the recipient population the federal policy was designed to serve. The historical eugenic mechanism the clawback parallels is the active provincial discouragement of disabled people from accessing resources available to non-disabled people. The historical version operated through institutional segregation. The contemporary version operates through the structural capture of federal transfer payments designed to lift disabled Albertans above the poverty line.

The FSCD Three-Year Wait

The Family Support for Children with Disabilities program funds respite, in-home support, therapy services, and equipment for Alberta children under eighteen with severe disabilities. It is the primary provincial mechanism through which families of disabled children access intervention services during the developmental years.¹²

In October and November 2024, Inclusion Alberta surveyed 540 families from 79 of Alberta's 87 electoral districts about their FSCD experience. The findings, published in January 2025, document an average wait of three years from application to full range of FSCD services. Eighty-five percent of families who had received the partial Family Support Services agreement that follows initial eligibility were still without a caseworker for the full Child Focused Services component. Fifty-eight percent of families reported negative effects from going without supports, including job loss, missed developmental windows, and measurable physical and mental health deterioration.¹³

The Auditor General of Alberta examined FSCD in 2022 and reported that caseworker training and oversight processes were inadequate and that families “may not receive the supports and services they qualify for.” The Government of Alberta received those findings in 2022. Caseworker caseloads have since been documented at four hundred families per worker against a standard reference of one hundred. Seven temporary caseworker contracts were not renewed in September 2024. The FSCD budget increase in Budget 2025 was 3.61 percent against a documented population-plus-inflation requirement of 7.3 percent — a real-terms reduction of approximately 3.69 percentage points.¹⁴

The structural effect is documented in the developmental literature on early intervention. The years between roughly two and five years of age constitute the period in which intervention services for developmental disability produce the largest measurable gains in functional outcomes. A three-year wait from application moves children past this window before services begin. A child diagnosed at four whose family applies the same day will not, on the documented average, receive full FSCD services until age seven — outside the window the program's own evidence base identifies as most consequential.¹⁵

The historical eugenic mechanism the FSCD wait parallels is the institutional intervention in disabled children's developmental trajectory documented in Chapter I. The historical version operated through institutionalization at the Provincial Training School and similar facilities, where children were separated from family and community on the asserted ground that institutional care was protective. The contemporary version operates through administrative wait time. The mechanism has changed from confinement to delay. The effect — the developmental trajectory the disabled child would otherwise have had is not the trajectory they receive — is the same.

PDD: Guardianship, Trusteeship, and the Unreported Waitlist

The Persons with Developmental Disabilities program supports adults eighteen and over with developmental disabilities of onset before age eighteen. It funds residential support, employment support, community access, and individualized care planning. PDD currently supports more than thirteen thousand adults in Alberta. The waitlist — the number of adults who are eligible for PDD but not receiving services — has not been publicly reported by the Government of Alberta in three years. The last published waitlist data are from 2021.¹⁶

Inclusion Alberta surveyed 540 PDD-connected individuals in late 2024 specifically because the Government would not release waitlist data. The findings, published in January 2025, document that seventy percent of respondents still need to meet with a caseworker to develop the outcome plan that determines their services. Forty-one percent of those who do receive supports report that the supports are

not helping. The PDD budget increase in Budget 2025 was 5.94 percent against the 7.3 percent population-plus-inflation requirement — a real-terms reduction of approximately 1.36 percentage points.¹⁷

The structural framework PDD operates within is the *Alberta Adult Guardianship and Trusteeship Act*. The Act establishes a substitute-decision-making framework for adults whose capacity to make personal and financial decisions is operationally impaired. The Act's framework conflicts with Article 12 (equal recognition before the law) of the CRPD, which the UN Committee on the Rights of Persons with Disabilities has identified, in General Comment No. 1, as requiring transition from substitute decision-making to supported decision-making across signatory jurisdictions.¹⁸

The historical eugenic mechanism the PDD framework parallels is the assertion of institutional authority over the lives of disabled adults. The Alberta Eugenics Board documented in Chapter I asserted authority to determine that disabled adults' reproductive lives were an appropriate site for state decision-making. The contemporary AGTA framework asserts authority over disabled adults' personal-and-financial decisions through a substitute-decision-making model that international human rights law has identified as requiring reform. The mechanism has changed from sterilization authority to substitute-decision-making authority. The operating premise — that disabled adults are appropriate subjects of external decision-making in ways non-disabled adults are not — is the operating premise of both periods.

Institutional Care: The Separation of Couples

The Alberta continuing care system, encompassing long-term care facilities (Continuing Care Home Type A) and designated supportive living facilities (Continuing Care Home Type B), operates with documented bed-availability constraints, facility-specialization patterns that distribute different levels of care across distinct facilities, and a fee-determination architecture based on couple's combined-income calculations. The combination produces a documented pattern in which spouses are forced to live apart when one requires institutional care that the other does not.¹⁹

In June 2019, CBC News reported the case of Martha and Willard Farnell of Calgary, Alberta. The Farnells had been married for sixty-six years. Following Willard's dementia diagnosis and admission to a long-term care facility approximately twenty minutes' drive from Martha's residence, the Alberta continuing care system did not provide a mechanism through which the couple could continue to share accommodation. The CBC report documented Sweden's policy position treating forced spousal separation in long-term care as inhumane, and the Netherlands "Living Apart Together" suites model that permits couples to live in separate but connected rooms or apartments within the same care facility.²⁰

The peer-reviewed medical literature documents that severe loneliness affects up to forty-two percent of long-term-care residents, against ten percent prevalence in older adults living in the community. The mortality and morbidity consequences of social isolation in this population are documented at scale.²¹

Ontario's long-term care regulatory framework, since January 1, 2018, has required every long-term care home in the province to have at least two beds available for spouses. Alberta has no equivalent requirement. The provincial Government's own acknowledged capacity need is ten thousand new continuing-care spaces over three years; documented commitments are at a small fraction of that figure.²²

The historical eugenic mechanism the institutional-care separation pattern parallels is direct. The cohabitation restrictions enforced in the historical residential institutions documented in Chapter I — the Provincial Training School, the network of mental hospitals — separated disabled adults from spouses, family, and community as a matter of institutional policy. The contemporary version separates senior spouses, one or both of whom is disabled, through bed-allocation and facility-specialization mechanisms that the policy architecture characterizes as administrative necessity. The mechanism has changed from policy prohibition to administrative allocation. The effect — partnered disabled adults are separated from their partners by the institutional system at the moment they most need continued partnership — is the same.

What This Chapter Establishes

The six mechanisms inventoried in this chapter are not the complete inventory of structural eugenics in contemporary Alberta. They are the inventory for which the primary-source record is the most complete. Each is documented in current Alberta legislation, regulation, or program policy. Each is verified against current government data or against survey work conducted by Inclusion Alberta, the Auditor General of Alberta, or the peer-reviewed medical literature. None of the six requires speculative inference; each operates as documented.

Three observations follow.

First, the six mechanisms operate together. The cohabitation rule restricts partnership. Sub-survival income restricts community participation. The CDB clawback nullifies federal poverty-reduction intervention. The FSCD wait restricts developmental trajectory. PDD guardianship restricts adult autonomy. Institutional care separation restricts continued partnership in aging. Taken together, they constitute an architecture in which disabled people are structurally separated from the relational, developmental, financial, and social conditions of full life across the lifecycle. The architecture is not the product of a single decision. It is the cumulative product of decisions taken over decades, each defensible in isolation, none requiring eugenic intent.

Second, the historical mechanisms documented in Chapter I have direct parallels in this inventory. The marriage prohibitions of the 1928–1972 period are paralleled by the cohabitation rule. The institutional segregation of the Provincial Training School period is paralleled by the income and housing structure that prevents community-based independent living. The institutional cohabitation restrictions are paralleled by the long-term-care separation pattern. The institutional authority over disabled adults’ lives asserted by the Eugenics Board is paralleled by the Adult Guardianship and Trusteeship Act substitute-decision-making framework. The developmental intervention of the historical institutionalization period is paralleled by the FSCD wait. The active discouragement of disabled family formation documented in the 1928–1972 record is paralleled by the integrated effect of all six contemporary mechanisms.

Third, none of the six mechanisms documented here was developed through a consultation process meeting the standard CRPD Article 4(3) requires. The cohabitation rule was filed by regulation on May 12, 2026, ten days after the Inclusion Alberta media release identifying Bill 12 as eroding disability rights, and was not preceded by consultation with disability community representative organizations on its

specific 88-percent mechanism. The CDB clawback was communicated to recipients by letter on July 2, 2025, without consultation. The Bill 12 framework producing the AISH-to-ADAP transition was passed on December 11, 2025 over the documented opposition of every major Alberta disability advocacy organization. The FSCD, PDD, and institutional-care frameworks operate within budget and policy cycles that do not, on the available record, include the consultation through representative organizations the Convention requires.

The mechanism has changed. The architecture has not.

Notes — Chapter III

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2. Assured Income for the Severely Handicapped General Regulation, AR 96/2026, filed May 12, 2026, in force July 2, 2026, section 8(4); Government of Alberta, Assisted Living and Social Services, “Alberta Disability Assistance Program Fact Sheet,” May 12, 2026.
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5. Government of Alberta, AISH and ADAP program documentation, 2025–2026; The Alberta Disability System Breakdown, *Income Crossover Analysis*, revised May 2026, on the transition payment expiring December 31, 2027.
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9. *Canada Disability Benefit Act*, SC 2023, c 9; Employment and Social Development Canada, Canada Disability Benefit program documentation, 2025.
10. Government of Alberta letter to AISH recipients, July 2, 2025 (notification of CDB treatment as non-exempt income); CBC News, “Federal disability benefit to be deducted from AISH payments,” Bellefontaine, M., February 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; Gillian Petit, “The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis,” *Canadian Tax Observatory*, April 22, 2026.
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12. Government of Alberta, Family Support for Children with Disabilities program documentation, alberta.ca.
13. Inclusion Alberta, *Experiences with Family Support for Children with Disabilities* (FSCD Survey Report), January 2025. 540 respondents from 79 of 87 Alberta electoral districts.
14. Auditor General of Alberta, *Report on FSCD*, 2022 (caseworker training and oversight findings); Hold My Hand Alberta / The Albertan News, “Some Alberta caseworkers that help disabled children have 100s of clients,” September 16, 2024; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
15. The developmental early-intervention window for children between approximately ages two and five is documented across the developmental disability literature. See, e.g., discussions in the FSCD Survey Report (Inclusion Alberta, January 2025) and the broader developmental pediatrics literature on intervention timing.

16. Inclusion Alberta / ALIGN, *PDD Current State: Critical and Urgent: Experiences with Persons with Developmental Disabilities*, January 2025; CBC News, “Provincial support delays are hurting disabled Albertans and families, reports find,” Bellefontaine, M., January 23, 2025.
17. Inclusion Alberta / ALIGN, *PDD Current State*; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
18. *Adult Guardianship and Trusteeship Act*, Statutes of Alberta, 2008 (provincial framework for substitute decision-making and trusteeship arrangements); Committee on the Rights of Persons with Disabilities, General Comment No. 1 (2014) on Article 12 of the CRPD (equal recognition before the law), document CRPD/C/GC/1.
19. Government of Alberta, continuing-care framework, including *Continuing Care Act* and related regulations; The Alberta Disability System Breakdown, *Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act*, Part One, Section 1.10.
20. Melissa Mancini, “Separating Couples in Long-Term Care: Some Nations Call It Inhumane, in Most of Canada It’s Routine,” CBC News, June 2, 2019, <https://www.cbc.ca/news/health/national-long-term-care-couples-housing-1.5159102>. The CBC report documents the Farnell case and comparator-jurisdiction approaches.
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